

Question 1

NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

History and Physical

Nurses' Notes

Vital Signs

Laboratory Results

Body System	Findings
Pulmonary	denies shortness of breath; reports discomfort in the lower left side of chest when taking a deep breath
Gastrointestinal	reports feeling abdominal fullness and is occasionally nauseated
Musculoskeletal	sustained an injury to the left rib cage after being struck by a mechanically pitched baseball in a batting cage last week; reports intermittent pain in the left shoulder rated 6/10 on the Numerical Rating Scale; light-headed; significant bruising to the shoulder; history of an orthoscopic repair to the left shoulder for a torn rotator cuff last year
Psychosocial	client has not felt well enough to attend baseball practice since the injury

The following 6 questions refer to this clinical scenario: NURS 1021 Unit 6 Case Study 2.

Which of the following assessment findings require immediate follow-up? Select all that apply.

- ☐ 1. lung sounds
- ☐ 2. shoulder pain
- ☐ 3. laboratory results
- ☐ 4. productive cough
- ☐ 5. abdominal assessment findings
- ☐ 6. pulse, respirations, and blood pressure
- ☐ 7. temperature and pulse oximetry reading

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History and PhysicalNurses' NotesVital SignsLaboratory Results

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☒ 3. laboratory results

☐ 4. productive cough

☒ 5. abdominal assessment findings

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☐ 7. temperature and pulse oximetry reading

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CORRECTScore: 5 / 5 points

Rationale:

Diminished breath sounds in the left lung fields (Option 1) require follow-up, since combined with the trauma history they raise concern for a hemothorax. Left shoulder pain (Option 2) requires follow-up because it can represent Kehr's sign, referred pain from diaphragmatic irritation caused by bleeding from a splenic injury, not just the client's old rotator cuff history. The laboratory results (Option 3) require follow-up: the hemoglobin of 9 g/dL and hematocrit of 27% are well below normal, consistent with ongoing blood loss, and the WBC of 19,000/mm³ reflects an acute inflammatory/traumatic response. The abdominal assessment findings (Option 5) require follow-up: LUQ bruising, tenderness, guarding, and dullness to percussion over the spleen's location are classic signs of splenic injury with possible intra-abdominal bleeding. The pulse, respirations, and blood pressure (Option 6) require follow-up because tachycardia (116), tachypnea (24), and hypotension (90/50) together are early signs of hypovolemic shock from blood loss. A productive cough (Option 4) is a nonspecific finding already reflected in the more specific lung sounds finding above, and the temperature and pulse oximetry reading (Option 7) are within normal limits and do not require follow-up.

Question 2

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

History and Physical

Nurses' Notes

Vital Signs

Laboratory Results

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Psychosocial	client has not felt well enough to attend baseball practice since the injury

➤ Which of the following issues is the client at risk of developing? **Select all that apply.**

- ☐ 1. stroke
- ☐ 2. hemothorax
- ☐ 3. bowel perforation
- ☐ 4. splenic laceration
- ☐ 5. pulmonary embolism
- ☐ 6. abdominal aortic aneurysm

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Question 2

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History and PhysicalNurses' NotesVital SignsLaboratory Results

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- ☐ 6. abdominal aortic aneurysm

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CORRECTScore: 3 / 3 points

Rationale:

Given blunt trauma to the left flank/rib cage with LUQ bruising, tenderness, diminished left breath sounds, tachycardia, and hypotension, the client is at risk for a splenic laceration (Option 4, the organ most often injured by blunt left-flank/rib trauma), a hemothorax (Option 2, given the diminished left lung sounds and chest discomfort with deep breathing), and bowel perforation (Option 3, a recognized complication of blunt abdominal trauma). A stroke (Option 1) is not a typical consequence of blunt torso trauma. A pulmonary embolism (Option 5) is not an immediate risk from this acute injury itself. An abdominal aortic aneurysm (Option 6) is a degenerative vascular condition that develops over time, not something caused by an acute traumatic injury.

Question 3

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

History and Physical

Nurses' Notes

Vital Signs

Laboratory Results

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Psychosocial	client has not felt well enough to attend baseball practice since the injury

➤ Complete the following sentence by choosing from the list of options.

The nurse should first address the client's

Select...▼

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Submit

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Question 3

COMPLETE

Case Study Screen 3 of 6

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Psychosocial	client has not felt well enough to attend baseball practice since the injury

➤ Complete the following sentence by choosing from the list of options.

The nurse should first address the client's

respiratory status

 .

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CORRECT

Score: 1 / 1 point

Rationale:

The client's respiratory status is the priority using the airway-breathing-circulation framework: diminished left breath sounds, chest discomfort with deep inspiration, tachypnea (RR 24), and borderline pulse oximetry all point to a possible hemothorax compromising oxygenation, which is more immediately life-threatening than addressing abdominal pain or reviewing laboratory results, both of which are important but secondary once the airway and breathing are confirmed to be adequately supported.

Question 4

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

History and Physical

Nurses' Notes

Vital Signs

Laboratory Results

Body System	Findings
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Psychosocial	client has not felt well enough to attend baseball practice since the injury

The nurse has reviewed the Nurses' Notes from 1000.

- For each potential order, click to specify whether the potential order is indicated or not indicated for the client.

Potential Orders	Indicated	Not Indicated
intravenous fluids	☐	☐
serum type and screen	☐	☐
chest percussion therapy	☐	☐
insertion of a nasogastric (NG) tube	☐	☐
administration of prescribed pain medication	☐	☐

Note: Each row must have only 1 response option selected.

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Question 4

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Case Study Screen 4 of 6

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History and Physical

Nurses' Notes

Vital Signs

Laboratory Results

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Submit

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CORRECT

Score: 5 / 5 points

Rationale:

Intravenous fluids are indicated to begin resuscitating a client with signs of hypovolemic shock from a splenic laceration. A serum type and screen is indicated to prepare for a likely blood transfusion and anticipated surgery. Chest percussion therapy is not indicated: it is contraindicated with a hemothorax and recent rib trauma, since it can worsen bleeding and cause significant pain. Insertion of an NG tube is indicated given the client's abdominal fullness and nausea and the likelihood of surgery, to decompress the stomach and reduce aspiration risk. Administration of prescribed pain medication is indicated to treat the client's reported 6/10 pain.

Question 5

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

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Nurses' Notes

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Psychosocial	client has not felt well enough to attend baseball practice since the injury

The nurse has reviewed the Nurses' Notes from 1030.

- Which of the following actions should the nurse take? **Select all that apply.**
- ☐ 1. Mark the surgical site.
- ☐ 2. Provide the client with ice chips.
- ☐ 3. Perform a medication reconciliation.
- ☐ 4. Obtain consent for surgery from the client.
- ☐ 5. Insert a peripheral venous access device (VAD).
- ☐ 6. Inform the client about the risks and benefits of the surgery.
- ☐ 7. Assess the client's previous experience with surgery and anesthesia.
- ☐ 8. Ask the client's parents to wait in the waiting room while the plan of care is discussed with the client.

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Question 5

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Case Study Screen 5 of 6

The nurse in the emergency department (ED) is caring for a 17-year-old male client.

History and Physical

Nurses' Notes

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CORRECT

Score: 4 / 4 points

Rationale:

Marking the surgical site, performing a medication reconciliation, inserting a peripheral VAD, and assessing the client's previous experience with surgery and anesthesia are all standard nursing pre-operative preparation tasks. Providing ice chips is inappropriate because the client needs to be NPO before surgery. Obtaining consent and informing the client about the risks and benefits of the surgery are the surgeon's/prescriber's responsibility, not the nurse's, and legally a minor client's parent or guardian must give consent, not the 17-year-old client. Asking the client's parents to leave while the plan of care is discussed is inappropriate for a minor client; parents/guardians should be included in care planning discussions, not excluded.

Question 6

NOT COMPLETE

Case Study Screen 6 of 6

The nurse has reviewed the Progress Notes from 0800.

➤ Click to highlight the findings below that indicate a worsening of the client's status.

Progress Notes

Client is postoperative day 3 after a splenectomy and is able to ambulate in the corridor 3 or 4 times daily with minimal assistance. Client has clear breath sounds bilaterally. Left-sided chest tube in place attached to a closed-chest drainage system. Tidaling of the water chamber noted on drainage system. Client refuses to use the incentive spirometer, stating it causes left-sided chest pain. Client is using prescribed patient-controlled analgesia (PCA) device maximally every hour and continues to have intermittent nausea and vomiting. Adequate urine output. Abdominal surgical incision site with dressing clean, dry, and intact with no erythema, edema, or drainage.

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Question 6

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Case Study Screen 6 of 6

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CORRECT

Score: 1 / 1 point

Rationale:

Refusing to use the incentive spirometer because of left-sided chest pain, combined with maximal PCA use every hour and continued nausea and vomiting, indicates worsening status: poorly controlled pain that prevents deep breathing raises the risk of atelectasis and pneumonia, and ongoing nausea/vomiting despite heavy analgesic use is not the expected postoperative trajectory by day 3. Ambulating with minimal assistance, clear bilateral breath sounds, an appropriately functioning (tidaling) chest tube drainage system, adequate urine output, and a clean, dry, intact incision without erythema, edema, or drainage are all reassuring, expected findings that do not indicate worsening.

The nurse is reviewing the orders of a client who has acute kidney injury.

➤ Which of the following orders should the nurse clarify?

- ☐ 1. computed tomography (CT) scan of the abdomen with intravenous contrast media
- ☐ 2. urine specimen for urinalysis
- ☐ 3. blood specimen for arterial blood gas (ABG)
- ☐ 4. referral to registered dietitian for parenteral nutrition evaluation

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Score: 1 / 1 point

Rationale:

Intravenous contrast media is nephrotoxic and can worsen kidney function in a client who already has acute kidney injury (AKI); this order should be clarified with the prescriber, who may choose an alternative imaging study or non-contrast CT. A urinalysis (Option 2) and an ABG (Option 3) are appropriate, expected diagnostic studies for evaluating AKI. A dietitian referral (Option 4) is appropriate, since nutritional needs commonly must be adjusted in AKI.

The nurse is planning a staff education program about caring for clients with restraints.

➤ Which of the following information should the nurse include?

- ☐ 1. "Restraints should be removed once during a shift to perform passive range-of-motion (ROM) exercises."
- ☐ 2. "Restraints should be secured to the side rails of the client's bed for quick release."
- ☐ 3. "Restraints require an order from the primary health care provider."
- ☐ 4. "Restraints may be used p.r.n. for clients who are confused."

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CORRECT

Score: 1 / 1 point

Rationale:

Restraints require a time-limited order from the primary health care provider (Option 3); they cannot be applied solely at nursing discretion. Restraints should be released and the limb assessed and repositioned at least every 2 hours, not just once per shift (Option 1). Restraints must be secured to the bed frame, never to a movable side rail (Option 2), which is a safety hazard if the rail is lowered. Restraint orders cannot be written as p.r.n. (Option 4); confusion alone does not automatically justify restraint use, and the least restrictive alternative must be tried first.

The nurse is caring for a client with active pulmonary tuberculosis (TB).

➤ Which of the following should the nurse include in the client's plan of care?

- ☐ 1. placing the client in a private room with the door open
- ☐ 2. putting a surgical mask on the client during transport to the radiology department
- ☐ 3. instructing the primary caregivers to wear surgical masks when caring for the client
- ☐ 4. instituting the standards for droplet precautions

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Rationale:

TB requires airborne precautions, and placing a surgical mask on the client for source control during transport outside the negative-pressure room (Option 2) is correct practice. The private room's door must stay closed, not open (Option 1), to maintain negative pressure. Caregivers need a fit-tested N95 (or higher-level) respirator, not a surgical mask (Option 3), to protect against airborne transmission. TB requires airborne precautions, not droplet precautions (Option 4).

The home-health nurse is teaching the parents of a 4-year-old client with impetigo.

➤ Which of the following information should the nurse include?

- ☐ 1. "Put a surgical mask on your child when around siblings."
- ☐ 2. "Cleanse the lesions with a povidone-iodine solution daily."
- ☐ 3. "Apply petroleum jelly to the lesions daily."
- ☐ 4. "Instruct your child not to use the same towels as siblings."

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CORRECT

Score: 1 / 1 point

Rationale:
Impetigo spreads by direct and indirect contact (including shared linens and towels), so not sharing towels with siblings (Option 4) is standard, correct contact-precaution teaching. A mask (Option 1) is unnecessary, since impetigo does not spread by the respiratory route. Daily cleansing is typically with gentle soap and water rather than povidone-iodine (Option 2). Petroleum jelly (Option 3) is not the standard treatment; prescribed topical or oral antibiotics are used instead, and an occlusive ointment is not the recommended lesion care.

The nurse has attended a staff education program about bioterrorism.

➤ Which of the following statements by the nurse would require follow-up?

- ☐ 1. "Botulism is transmitted by ingestion of contaminated canned foods."
- ☐ 2. "Hemorrhagic fever is spread by direct contact with blood or body fluids."
- ☐ 3. "Anthrax is spread through direct contact with the bacteria and its spores."
- ☐ 4. "Bubonic plague is transmitted from person to person via airborne droplets."

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Rationale:

Bubonic plague (Option 4) is transmitted primarily through the bite of infected fleas, not person-to-person via airborne droplets; it is pneumonic plague, a different clinical form, that can spread person to person via respiratory droplets, so this statement is incorrect and requires follow-up. Botulism from contaminated canned foods (Option 1), hemorrhagic fever spread by contact with blood/body fluids (Option 2), and anthrax spread through direct contact with the bacteria and spores (Option 3) are all accurate statements that do not require follow-up.

The nurse observes a coworker who is assessing a client's thoracic expansion.

➤ Which of the following would indicate that the coworker is using the correct assessment technique?

- ☐ 1. percussion from the apex of the scapula downward on each side
- ☐ 2. placement of the hands flat on the back with the thumbs at the level of the tenth ribs pointing to the spine, then asking the client to inhale
- ☐ 3. measurement of the anteroposterior diameter of the chest
- ☐ 4. placement of the palms at the level of the tenth ribs with thumbs pointing to the xiphoid process, then asking the client to inhale

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Score: 1 / 1 point

Rationale:

Posterior thoracic (respiratory) excursion is assessed by placing the hands flat against the client's back with the thumbs at about the level of the tenth ribs pointing toward the spine, then observing the thumbs move symmetrically apart as the client inhales (Option 2). Percussion from the scapula downward (Option 1) assesses lung resonance/diaphragmatic excursion, a different parameter, not chest wall expansion. Measuring the anteroposterior diameter (Option 3) assesses chest configuration (for example, a barrel chest), not excursion during breathing.

The nurse at a health fair is talking with a client who is in perimenopause and is experiencing hot flashes.

➤ Which of the following lifestyle modifications would be appropriate for the nurse to recommend?

- ☐ 1. increasing fluid intake
- ☐ 2. exercising daily
- ☐ 3. decreasing sodium intake
- ☐ 4. wearing clothing in layers

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CORRECT

Score: 1 / 1 point

Rationale:
Wearing clothing in layers (Option 4) is a standard, practical recommendation for managing hot flashes, since layers can be quickly removed when a hot flash occurs and added back as it resolves. Increasing fluid intake (Option 1) and decreasing sodium intake (Option 3) are general health measures not specifically targeted at hot flash management. Daily exercise (Option 2) supports overall menopausal health but is not the specific, targeted recommendation for managing an acute hot flash episode the way dressing in layers is.

The nurse in a community-based setting is teaching clients over 65 years of age about health promotion activities.

➤ Which of the following information should the nurse include?

- ☐ 1. "Purchase all of your prescribed medications at the same pharmacy."
- ☐ 2. "Schedule an appointment for a vision screening every 3 years."
- ☐ 3. "Participate in daily aerobic exercises for 60 minutes."
- ☐ 4. "Increase your intake of fat-soluble vitamins."

Submit

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CORRECT

Score: 1 / 1 point

Rationale:

Using a single pharmacy (Option 1) allows the pharmacist to screen all of a client's medications together for interactions and duplications, an important safety measure for older adults. Vision screening (Option 2) should generally occur annually for this age group, not every 3 years. Recommending 60 minutes of daily aerobic exercise (Option 3) exceeds standard general guidelines (about 150 minutes per week of moderate activity) and may not be realistic or safe as a blanket recommendation. Increasing fat-soluble vitamin intake (Option 4) is not a general health-promotion recommendation, and excess fat-soluble vitamins (A, D, E, K) can accumulate to toxic levels.

The nurse is screening clients for those at increased risk for developing cancer.

➤ At **highest** risk for developing leukemia is the client who

- ☐ 1. received more than 3 blood transfusions
- ☐ 2. has a magnetic resonance imaging (MRI) scan annually
- ☐ 3. has polycythemia vera and requires phlebotomy treatments
- ☐ 4. had colon cancer and received chemotherapy treatments

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CORRECT

Score: 1 / 1 point

Rationale:
Certain chemotherapy agents, particularly alkylating agents and topoisomerase II inhibitors commonly used to treat colon cancer, are well-established causes of treatment-related (secondary) leukemia, making this client's risk the highest of those listed. Multiple blood transfusions (Option 1) are not an established leukemia risk factor. MRI (Option 2) uses no ionizing radiation and is not a cancer risk factor, unlike CT or x-ray imaging. Polycythemia vera (Option 3) carries some increased risk of transformation to acute leukemia over time, but this risk is generally smaller in magnitude than the risk associated with leukemogenic chemotherapy.

The nurse is caring for an older adult client in the postoperative period.

➤ The nurse should know that this client, compared with younger clients in the postoperative period, will have an **increased** need for

- ☐ 1. oral hygiene
- ☐ 2. analgesics
- ☐ 3. high-calorie foods
- ☐ 4. early mobilization

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CORRECT

Score: 1 / 1 point

Rationale:
Older adults have an increased need for early mobilization after surgery (Option 4) because they are at substantially higher risk than younger clients for complications of immobility such as pneumonia, venous thromboembolism, pressure injury, and functional decline, and they recover mobility more slowly. Oral hygiene (Option 1) is important for all postoperative clients but is not specifically increased by age. Analgesic needs (Option 2) are often lower, not higher, in older adults due to altered pharmacokinetics and increased sensitivity to side effects. Caloric needs (Option 3) are generally decreased, not increased, in older adults due to a lower metabolic rate.

The nurse is planning a staff education program about the prevention of urinary tract infections (UTIs) in children.

➤ Which of the following information should the nurse include? **Select all that apply.**

- ☐ 1. "Teach the child to perform Kegel exercises."
- ☐ 2. "Encourage the child to empty the bladder completely."
- ☐ 3. "Encourage the child to maintain an adequate fluid intake."
- ☐ 4. "Teach the child how to properly cleanse the perineal area."
- ☐ 5. "Offer the child noncarbonated, decaffeinated beverage choices."

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CORRECT

Score: 4 / 4 points

Rationale:
Complete bladder emptying (Option 2) prevents urinary stasis that allows bacteria to multiply. Adequate fluid intake (Option 3) helps flush bacteria from the urinary tract. Proper perineal cleansing technique, such as front-to-back wiping (Option 4), prevents introducing bacteria into the urethra. Noncarbonated, decaffeinated beverages (Option 5) avoid bladder irritants that carbonation and caffeine can cause. Kegel exercises (Option 1) strengthen pelvic floor muscles for continence and are not a standard pediatric UTI prevention teaching point.

The nurse is teaching the family member of a client with moderate Alzheimer's disease (AD).

➤ Which of the following interventions should the nurse include in the teaching? **Select all that apply.**

- ☐ 1. Use distraction when the client becomes agitated.
- ☐ 2. Place calendars within clear view of the client.
- ☐ 3. Use short, simple sentences and provide step-by-step instructions for the client.
- ☐ 4. Avoid reminiscing with the client about past experiences in order to avoid feelings of loss and loneliness.
- ☐ 5. Encourage the client to participate in a daytime exercise program to promote restful sleep at night.

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CORRECT

Score: 4 / 4 points

Rationale:

Distraction (Option 1) is a standard de-escalation technique for agitation in AD. Calendars (Option 2) support reality orientation. Short, simple sentences with step-by-step instructions (Option 3) match the client's cognitive processing ability. A daytime exercise program (Option 5) helps regulate the sleep-wake cycle and can reduce sundowning and improve nighttime rest. Reminiscence about long-term memories (Option 4) should be encouraged, not avoided, since long-term memory is often preserved in AD and reminiscing is a validating, anxiety-reducing activity.

The nurse is preparing to administer a unit of packed red blood cells (PRBCs) to a client.

➤ Which of the following actions should the nurse take?

- ☐ 1. Assess the client's recent urine output.
- ☐ 2. Prime a Y-tubing blood administration set with lactated Ringer's solution.
- ☐ 3. Ensure that the client has a peripheral venous access device (VAD) that is 24-gauge or larger.
- ☐ 4. Verify with another nurse that the client's room number is on both the blood product label and the client's identification band.

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CORRECT

Score: 1 / 1 point

Rationale:

Blood is administered through a VAD of at least 24-gauge (that is, 24-gauge or a larger-bore catheter such as 22, 20, or 18-gauge) to allow safe infusion of the product (Option 3). Blood administration sets must be primed with 0.9% sodium chloride only, never lactated Ringer's solution (Option 2), because the calcium in Ringer's solution can cause the blood product to clot. Room number is never an acceptable client identifier for verifying a blood product against the client's identification band (Option 4); a full name and a unique identifier such as a medical record number must be used instead. Assessing recent urine output (Option 1) is not a required immediate pre-transfusion action; it is more relevant to monitoring after the transfusion begins.

The nurse is assessing the coping strategies of a client who had a myocardial infarction (MI) 3 days ago.

➤ Which of the following statements by the client would indicate ineffective coping?

- ☐ 1. "I know that stopping smoking will be difficult."
- ☐ 2. "I plan to attend a cardiac rehabilitation support group."
- ☐ 3. "I have trouble believing this has really happened to me."
- ☐ 4. "I have let down my family because I will not be able to financially support them any longer."

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CORRECT

Score: 1 / 1 point

Rationale:

Assuming, only 3 days after an MI, that the client will permanently be unable to work and support their family (Option 4) reflects catastrophic thinking and hopelessness not grounded in the client's actual prognosis, indicating ineffective coping and warranting further assessment for depression or excessive guilt. Acknowledging that quitting smoking will be difficult (Option 1) and planning to attend cardiac rehabilitation (Option 2) both reflect realistic, adaptive coping and engagement with recovery. Some difficulty believing the MI happened (Option 3) reflects an early, commonly expected stage of processing a sudden diagnosis rather than clearly ineffective coping at only 3 days out.

The hospice nurse has taught an in-home caregiver about comfort care for a client at the end of life.

➤ Which of the following statements by the caregiver would require follow-up?

- ☐ 1. "I have been applying petroleum jelly to keep the client's lips moist."
- ☐ 2. "I have been offering healthy foods frequently to keep up the client's strength."
- ☐ 3. "A blowing fan seems to be less anxiety-producing for the client than an oxygen mask."
- ☐ 4. "Sitting upright seems to reduce the client's noisy breathing more than lying down in the bed."

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CORRECT

Score: 1 / 1 point

Rationale:
Frequently offering food to "keep up strength" (Option 2) requires follow-up, because appetite naturally declines at the end of life and end-of-life care focuses on comfort, not on forcing intake to build strength; frequent food offers can cause discomfort or distress. Petroleum jelly for dry lips (Option 1) is an appropriate comfort measure. A fan blowing air on the face (Option 3) is a well-supported way to relieve the sensation of dyspnea, often with less associated anxiety than an oxygen mask. Sitting upright to reduce noisy breathing ("death rattle") (Option 4) is an accurate, appropriate observation, since upright positioning can improve secretion drainage.

The nurse is witnessing the client's signature on a consent form.

➤ Which of the following conditions should the nurse recognize must be met to ensure the consent is valid? **Select all that apply.**

- ☐ 1. The client gave consent voluntarily.
- ☐ 2. The client received adequate disclosure.
- ☐ 3. The consent form is witnessed by 2 health care professionals.
- ☐ 4. The client understands the scheduled procedure or treatment.
- ☐ 5. The consent form is signed within 24 hours of the scheduled procedure or treatment.

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CORRECT

Score: 3 / 3 points

Rationale:

Valid informed consent requires that it be given voluntarily (Option 1), that the client received adequate disclosure of the risks, benefits, and alternatives (Option 2), and that the client demonstrates understanding of the procedure or treatment (Option 4). Consent forms typically require only 1 witness signature, not 2 health care professionals (Option 3). There is no universal requirement that consent be signed within a specific 24-hour window of the procedure (Option 5).

The nurse is talking with a client who has been sexually assaulted. The client states, "I never should have walked home late at night. I am to blame for what has happened to me."

➤ Which of the following would be an appropriate response for the nurse to make? **Select all that apply.**

- ☐ 1. "The police officers who brought you into the hospital will be with you during this interview."
- ☐ 2. "You should take a warm, calming shower in order to feel more relaxed."
- ☐ 3. "You did the best you could in very difficult circumstances."
- ☐ 4. "Sometimes the victim's behavior causes the violence."
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CORRECT

Score: 2 / 2 points

Rationale:

"You did the best you could in very difficult circumstances" validates the client and counters self-blame. "You are safe here" provides appropriate reassurance and helps establish a sense of safety. Having police remain present during a sensitive forensic/medical interview is not standard best practice; the client should have control over who is present, generally a support person or advocate of the client's choosing. Recommending a shower before a forensic examination is incorrect and harmful, since showering destroys evidence needed for the examination. Suggesting that "the victim's behavior causes the violence" is victim-blaming and is never an appropriate statement.

The nurse is planning care for a client with moderate Alzheimer's disease (AD).

➤ Which of the following interventions should the nurse include in the client's plan of care? **Select all that apply.**

- ☐ 1. Establish a daily routine for the client.
- ☐ 2. Assist the client to void every 2 hours.
- ☐ 3. Introduce self upon interacting with the client.
- ☐ 4. Display a clock and calendar in the client's room.
- ☐ 5. Keep the client's television on during the day to distract the client.

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CORRECT

Score: 4 / 4 points

Rationale:

A consistent daily routine (Option 1) reduces confusion and anxiety in AD. Scheduled toileting every 2 hours (Option 2) is a proactive continence strategy. Reintroducing oneself at each interaction (Option 3) accommodates impaired recognition memory and reduces fear or confusion. A clock and calendar (Option 4) support reality orientation. Keeping the television on continuously (Option 5) is not appropriate; excess background stimulation tends to increase confusion and agitation in AD rather than help, and a calm, low-stimulation environment is generally preferred.

A parent is discussing with the nurse the behaviors of a 4-year-old child following the death of a grandparent.

➤ The nurse should understand that the child may be experiencing dysfunctional grieving if the parent reports that the child

- ☐ 1. conducts mock funerals with stuffed animals
- ☐ 2. refuses to go to sleep at night
- ☐ 3. continues to talk about the grandparent coming to visit
- ☐ 4. asks to play with the grandparent while at the cemetery

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CORRECT

Score: 1 / 1 point

Rationale:
Persistent refusal to sleep at night (Option 2) reflects ongoing anxiety and dysregulation that goes beyond the expected, developmentally typical grief responses of a preschooler and can indicate the grieving process is becoming dysfunctional. Reenacting funerals through play (Option 1), continuing to talk about the deceased returning (Option 3), and asking to play with the grandparent at the cemetery (Option 4) all reflect the magical thinking and play-based processing that are normal, expected features of how preschool-age children process death, since children this age do not yet fully grasp its permanence.

The nurse has taught a client who has been ordered a low-sodium diet about appropriate food choices.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching?

- ☐ 1. "I will eat steamed, fresh broccoli with herbs and spices for an evening meal."
- ☐ 2. "I will add cottage cheese and other dairy products to my daily diet."
- ☐ 3. "I am glad I can still enjoy eating cereals, such as bran flakes with raisins."
- ☐ 4. "I am glad I can eat lean meats daily because I eat ham sandwiches for an afternoon meal."

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CORRECT

Score: 1 / 1 point

Rationale:
Fresh steamed vegetables seasoned with herbs and spices instead of salt (Option 1) is exactly the substitution pattern taught for a low-sodium diet. Cottage cheese (Option 2) and many processed breakfast cereals such as bran flakes (Option 3) are common hidden-sodium sources that clients often mistakenly consider low-sodium choices. Ham (Option 4) is a cured, processed meat that is very high in sodium and is not an appropriate substitute for a fresh lean meat on a low-sodium diet.

The nurse is caring for a client who had a left modified radical mastectomy. The client received discharge instructions for performing range-of-motion (ROM) exercises on her left arm.

➤ Which of the following, if reported by the client on her return visit to the clinic, would indicate to the nurse that the instructions have been followed correctly?

- ☐ 1. regular squeezing of a tennis ball in her left hand
- ☐ 2. placing her left palm against a wall and "climbing" the wall with the left fingers
- ☐ 3. carrying light hand weights while walking 1 mile every other day
- ☐ 4. performing isometric exercises with both arms extended

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CORRECT

Score: 1 / 1 point

Rationale:
"Wall climbing" with the fingers of the affected arm (Option 2) is the classic, specifically taught postmastectomy exercise for restoring shoulder range of motion. Carrying weights with the affected arm (Option 3) is generally discouraged early in recovery because of the risk of lymphedema, particularly after axillary lymph node involvement. Squeezing a tennis ball (Option 1) exercises the hand but is not the specific shoulder ROM exercise taught for this surgery. Isometric exercises with the arms extended (Option 4) are not part of the standard early postmastectomy ROM teaching.

The nurse is planning care for a client who has expressive aphasia after a left-sided stroke.

➤ Which of the following statements by the client's spouse would indicate a correct understanding of the client's communication abilities and interaction needs? **Select all that apply.**

- ☐ 1. "My spouse's response of 'fine' when I asked how the day has been may or may not be what my spouse meant to communicate."
- ☐ 2. "I can anticipate what my spouse wants to say, so I complete my spouse's sentences to make communication quicker."
- ☐ 3. "I will purchase a picture board to help my spouse express common needs, thoughts, and feelings when communication is difficult."
- ☐ 4. "My spouse's angry response when we have a conversation makes me hesitant to try further communication."
- ☐ 5. "I have arranged for my spouse to meet with a speech therapist twice each week to improve communication skills."

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CORRECT

Score: 3 / 3 points

Rationale:

Recognizing that a simple response like "fine" may not accurately reflect what the client intends to say (Option 1) reflects correct understanding of expressive aphasia, in which producing intended language is impaired. Obtaining a picture communication board (Option 3) and arranging regular speech therapy (Option 5) are both appropriate, standard interventions for expressive aphasia. Habitually finishing the client's sentences (Option 2) is not recommended, since it can be frustrating and does not reliably reflect what the client intends to say; the client should be given time to communicate. Becoming hesitant to attempt further communication because of the client's frustration (Option 4) is an avoidant response, not an appropriate coping strategy.

The nurse is caring for a client who is in Buck traction.

➤ Which of the following would require **immediate** intervention?

- ☐ 1. A pillow is placed under the knee.
- ☐ 2. The foot is 2 in (5 cm) away from the foot plate.
- ☐ 3. The weights attached to the pulley are 6 in (15 cm) from the floor.
- ☐ 4. A pillow is placed under the lower leg with the heel off the bed.

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CORRECT

Score: 1 / 1 point

Rationale:
Buck traction requires the affected leg to remain straight and in line with the pull of the traction; placing a pillow under the knee (Option 1) flexes the knee, misaligns the traction, and undermines its therapeutic effect, requiring immediate intervention. The foot being a short distance from the foot plate (Option 2) is an acceptable, expected finding. Weights hanging freely a few inches off the floor (Option 3) is the correct, expected setup. A pillow supporting the length of the lower leg while keeping the heel off the bed (Option 4) is an appropriate technique to prevent heel pressure injury without disrupting alignment.

The nurse has taught the adult child caregiver of a client with moderate Alzheimer's disease (AD) about home care.

➤ Which of the following statements by the adult child would indicate a correct understanding of the teaching?

- ☐ 1. "I will only allow my parent to smoke while my parent is outdoors."
- ☐ 2. "I will place a picture on the bathroom door to indicate which room is the bathroom."
- ☐ 3. "I will encourage family members to visit in large groups to keep my parent interested in the conversation."
- ☐ 4. "I will encourage my parent to take walks in the park when the weather permits to get the exercise needed."

Submit

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The nurse has taught the adult child caregiver of a client with moderate Alzheimer's disease (AD) about home care.

➤ Which of the following statements by the adult child would indicate a correct understanding of the teaching?

- ☐ 1. "I will only allow my parent to smoke while my parent is outdoors."
- ☒ 2. "I will place a picture on the bathroom door to indicate which room is the bathroom."
- ☐ 3. "I will encourage family members to visit in large groups to keep my parent interested in the conversation."
- ☐ 4. "I will encourage my parent to take walks in the park when the weather permits to get the exercise needed."

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CORRECT

Score: 1 / 1 point

Rationale:

A picture on the bathroom door (Option 2) is a well-established, effective wayfinding aid for clients with AD who have difficulty with word/symbol recognition. Location alone does not address the core safety issue with smoking (Option 1); a client with AD needs direct supervision while smoking regardless of setting, due to fire-safety risk and impaired judgment. Large group visits (Option 3) tend to overstimulate and confuse clients with AD; small, quiet visits are generally recommended instead. Unsupervised walks in the park (Option 4) raise a significant wandering and safety risk for a client with AD.

The nurse is teaching a client newly diagnosed with diverticulosis.

➤ Which of the following information should the nurse include?

- ☐ 1. "Limit your daily fluid intake to 2 L to avoid bloating."
- ☐ 2. "You may be prescribed a bulk-forming laxative."
- ☐ 3. "Limit your intake of dairy products such as milk and yogurt."
- ☐ 4. "You should avoid consuming cooked vegetables."

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CORRECT

Score: 1 / 1 point

Rationale:

A bulk-forming laxative such as psyllium (Option 2) is commonly prescribed for diverticulosis to promote regular, soft bowel movements and reduce intraluminal pressure. Diverticulosis management encourages adequate, not limited, fluid intake (Option 1) alongside a high-fiber diet to prevent constipation. There is no standard indication to limit dairy products (Option 3). Cooked vegetables (Option 4) are a recommended source of fiber and are not restricted; the older recommendation to avoid seeds, nuts, and popcorn is now considered outdated and not evidence-based, and cooked vegetables were never part of that restriction.

The nurse is preparing to administer lorazepam 2 mg, IV, now to a client who is scheduled for surgery in 30 minutes. The nurse is unfamiliar with the dosage for the medication.

➤ Which of the following actions should the nurse take **next**?

- ☐ 1. Check the medication dosage in a medication reference source.
- ☐ 2. Ask another nurse whether the prescribed dose is a safe dose.
- ☐ 3. Clarify that the dose is correct with the primary health care provider.
- ☐ 4. Contact the pharmacist to verify the safe dosage range for the medication.

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CORRECT

Score: 1 / 1 point

Rationale:

The pharmacist is the medication expert and the most authoritative, reliable resource for verifying a safe dosage range, especially in a time-sensitive situation (Option 4). Checking a general reference (Option 1) is reasonable but less authoritative than a pharmacist consultation for verifying a specific order. Asking a peer nurse (Option 2) is not an authoritative safety check. Contacting the prescriber (Option 3) is appropriate if there is concern the order itself is wrong, but the nurse's stated problem here is simply unfamiliarity with the medication, which the pharmacist is best positioned to resolve.

The nurse is caring for a client who is receiving a high dose of a phenothiazine.

➤ When evaluating the client for a life-threatening syndrome related to the medication, it would be a priority for the nurse to report

- ☐ 1. dry mouth
- ☐ 2. orthostatic hypotension
- ☐ 3. fever
- ☐ 4. photophobia

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CORRECT

Score: 1 / 1 point

Rationale:
Fever is a cardinal sign of neuroleptic malignant syndrome (NMS) (Option 3), a rare but life-threatening reaction to phenothiazines and other antipsychotics that also includes muscle rigidity, altered mental status, and autonomic instability; it must be reported immediately. Dry mouth (Option 1) and orthostatic hypotension (Option 2) are common, expected anticholinergic and alpha-blocking side effects of phenothiazines, not signs of NMS. Photophobia (Option 4) is not a typical finding of either routine phenothiazine side effects or NMS.